



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : امل حسن عبدالفتاح السيد
المحافظة : الجيزة
الحى / المدينة : ابوالنمرس
رقم البطاقة : 27602172402421 رقم التليفون : 01144530915
العنوان : ++++++
العمر (تاريخ الميلاد) : 17-02-1976 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 23-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: امل حسن عبدالفتاح السيد National ID: 27602172402421	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الى Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Patient Name	: الأمل محمد عبد السلام	Request No.	: 135818
Patient No.	: 27602172402421	Request Date	: 30/12/2021
Path No.	: 11/2022	Verification Date	: 10/01/2022
Sex	: Female	Financial Category	: دورهم دار السلام - مريض
Age	: 45 Y	Department	: Out Patient
Referring Doc	:	Registered by	: manwa mohamed youss

Pathology Report

- Resection Margins Free
- Histologic response to therapy N/A
- Additional pathologic findings Focal microcalcification, moderate dysmoplasia.

Diagnosis:

Left breast cancer, Wide local excision and sentinel lymph nodes biopsy

- Invasive duct carcinoma , grade II
- Free all margins
- Free all SLNs (0/6)
- TNM: pT2c, pNO, pMx.

Prof. Dr. Gamal Abd Elatty

Dr. Walaa Ghonaim

Dr. Esraa Magdy

Verified by :
walaa ghonaim

Printing Date: 11-01-2022 11:37:25

Page 1 of 1

Printing By : Dr. Eng. S



Patient Name: Amal Hassan Abdel Fattah Al Sayed	Date : 15/05/2023
Exam : MAMMOGRAPHY BOTH	Age : 47 Year(s)
Medical ID: 4644/2021	Pat. ID: 33974

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

- Left breast conserving therapy

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

Left breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Right Breast:

- Scattered fibro glandular density (ACR B) with adenosis pattern.
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary L.N.

Opinion:

- Post left BC'S status showing no evidence of recurrence (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Signature Dr. Peter Nashat

15/05/2023 1:25 PM

Patient Name : 27602172402421
 Patient No. : 27602172402421
 Path No. : 2416/21
 Sex : Female
 Age : 45 Y
 Referring Doctor :
 Request No. : 130098
 Request Date : 13/10/2021
 Verification Date : 25/10/2021
 Financial Category : Out Patient
 Department :
 IHC No. :

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass

Nature of the specimen: Biopsy
 Gross Examination: Four gray/whitish tissue cores, 0.3cm up to 1.3cm long. Totally submitted.
 Procedure: Tricut
 Localization: Left
 Immunohistochemistry technique:
 Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (SP1), PR clone (1E2), HER-2 CLONE (4B5), & K67 (30-9)

Microscopic examination:
 Histological type: Invasive duct carcinoma
 Grade : G II
 Angiolymphatic: No
 Ductal carcinoma in situ: No

Diagnosis:
 Left breast, Core biopsy
 Invasive duct carcinoma, grade II

Biomarkers:
 ER: Strong 90% nuclear reaction (Allred score 8/8)
 PR: Strong 90% nuclear reaction (Allred score 8/8)
 HER2: No membrane reaction (score 0)
 Ki67: 15% nuclear reaction

Prof. Dr. Anaj Hareedy
 Dr. Mona Mohamed

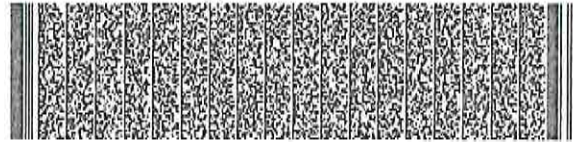


٢٠٢٥/٠٢ ٢٧٦٠٢١٧٢٤٠٢٤٢١

ربة منزل



أنثى مسلمة متروجة
سيد ربيع عبدالفتاح السيد
البطاقة سارية حتى ٢٠٣٢/٠٢/٠٧



بسم الله الرحمن الرحيم
بطاقة تحقيق الشخصية



امل
حسن عبدالفتاح السيد
قرية ترسا
مركز ابو التمرس - الجيزه



٢٧٦٠٢١٧٢٤٠٢٤٢١
LA1211394

Patient's Name:

Amal Hassan Abdel Fattah

Date:

2026-06-23

Referred by:

Prof.Dr. no doctor
Goustave Roussey International Egypt

Hospital:

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:
sonography revealed:

Clinical summary: Post left CBS and ALAND followed by radiotherapy, routine follow up study.

Findings:

Right breast

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast

- Heterogenously dense breast parenchyma that may obscure small lesions, or suspicious masses, asymmetry , or suspicious
- Thickened skin and coarsen trabeculae.
- Outer central architectural distortion with no speculations, masses, asymmetry, or suspicious microcalcifications.
- **Ultrasound** demonstrated 3 o'clock area of parenchymal disruption underneath the operative scar with no solid , cystic lesions or fluid collections.
- Thickened skin reaching 5 mm and coarsen trabeculae.
- Normal nipple/areola complex.
- Normal left axilla. No pathological infra or supra-clavicular lymph nodes

Conclusion:-

Right breast (BIRADS 1): Normal study.

Left breast (BIRADS 2): Post-therapeutic sequelae.

Recommendation:-

For annual follow up.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations

BIRADS 0: Non-informative study.

BIRADS 1: Normal study, routine screening

BIRADS 2: Benign findings, routine screening

BIRADS 3: Probably benign, for close follow up.

Suspected Cancer Risk

0 %

0 %

2 %



संस्कृत-विभाग

C.55-1-55

Name	Colin Campbell
Age	45 yrs.
M/R/N	positive
ID	CVN-CIVC-CCCI
Co	free
morbidity	

Cl

Clinical Data

Bx:	IDC, GII
SM:	11/28/1 Lt breast why 2-5 x 2-8 cm
IHC:	
ER:	+
PR:	+
Her2:	negative
KiTV:	
Mets Workup:	

$$T_{N\frac{1}{2}M}$$

COMMENT:

- More 2x1.5x1 cm (laminated A)
- we SLNB 0/6

MDT OUTCOME & PLAN

Adj TC x 4, G SF. & D^{7D}₍₅₎ (7)

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
<i>aslam</i>		<i>Prasad</i>	

Report of Contrast Abdominal CT Scan
Date: 10/10/2021

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST
ABDOMEN & PELVIS REVEALED:-**

1. Chest:

- * Left breast irregular enhancing parenchymal mass is seen measuring 2.5x 1.7 cm.
- * Middle lobar, tiny pulmonary nodule is seen measuring 4 mm, likely insignificant.
- * Otherwise, No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- * No pathological hilar or mediastinal lymph nodes.
- * No evidence of pleural effusion.
- * Normal cardiac size. No pericardial effusion detected.
- * Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- * The liver enlarged with homogenous CT texture. No focal lesions or contour irregularities.
- * Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- * No intra-hepatic or extra-hepatic biliary radicles dilatation.
- * Normal size & parenchymal texture of the spleen with no focal lesions seen.
- * Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- * Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- * Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- * Slightly bulky uterus.
- * Normal appearance of the other pelvic organs.
- * No abdominal or pelvic lymphadenopathy or collections.
- * No suspicious lytic or sclerotic bone lesions.

OPINION:

- * Left breast, LOQ irregular enhancing parenchymal mass lesion.
- * No definite pulmonary, hepatic or osseous metastatic deposits.

Dr. Peter Mashadi, MSc

Dr. Yousang Park, MD

Patient Name : امل حسن عبد الفتاح
Patient No : 4644-2021
Sex : Female Age : 48 Y
Financial Cate : نفقة الزوجية - جواز سفر روسي
Department : Out Patient
Referring Doc :

Lab No. : 13007001
Request Date : 21-07-2025 15:54
Verification Date : 21-07-2025 16:45
Sample Type : EDTA Blood
Test Status : Routine
Analyzers : XN350_hermi

Complete Blood Picture

Test	Result	Unit	Ref. Range
Hemoglobin	<u>10.9</u>	g/dL	12.0 - 16.0
RBCs	<u>3.8</u>	$10^6/\mu\text{L}$	4.0 - 6.5
Haematocrit (PCV)	<u>31.5</u>	%	36.0 - 50.0
MCV	83.1	FL	76.0 - 101.0
MCH	28.8	pg	26.0 - 32.0
MCHC	<u>34.6</u>	g/dL	31.5 - 34.5
RDW-CV	12.9	%	11.6 - 14.0
PLT	212	$10^3/\mu\text{L}$	150 - 450
WBCs.Total	4.8	$10^3/\mu\text{L}$	4.0 - 11.5

Differential Leucocytic Count:

	Relative	Absolute	
Neutrophils%	65.0	3.12	/cmm 2.00 - 7.00
Lymphocytes%	27.2	<u>1.31</u>	/cmm 1.50 - 3.50
Monocytes %	6.2	0.30	/cmm 0.20 - 1.00
Eosinophils %	1.2	0.06	/cmm Up to 0.50
Basophils %	0.4	0.02	/cmm Up to 0.10

Signature

Dr./Manal Mokhtar

Patient No. : 4644-2021
الاستشارة / امل حسن عبد الفتاح
Patient Name :
Sex : Female Age: 48 Y
Financial Cate: نفقة الزوجة - جواز سفر روسي
Department : Out Patient
Referring Doc :

Request No : 13007001
Request Date : 21-07-2025 15:54
Verification Date : 21-07-2025 17:18
Sample Type : Serum
Test Status : Routine
Analyzers : biolis_hermel

Clinical Chemistry Report

Test	Result	Unit	Ref. Range	
ALT (SGPT)	20.8	U/l	Up to 31.0	)
AST (SGOT)	22.0	U/l	Up to 31.0	)
Serum Creatinine	1.0	mg/dl	0.6 - 1.2	)
Uric acid	5.3	mg/dl	2.6 - 6.0	)
Urea	27.5	mg/dl	17.0 - 43.0	)
Calcium Ionized (Ca++)	1.14	mmol/l	1.10 - 1.30	)
Bilirubin Total	0.41	mg/dl	0.10 - 1.20	)
Bilirubin Direct	0.14	mg/dL	Up to 0.20	)

Signature

Dr./Manal Mokhtar

GUSTAVE / ROUSSY

INTERNATIONAL
ACGYPTE

Patient Name: Amal Hassan Abdel Fattah Al		Date : 21/07/2025
Sayed		Age : 49 Year(s)
Exam : ultrasound TRANS VAGINAL		Pat. ID: 33974
Medical ID:		

Trans-vaginal ultrasound scanning revealed:-

- Uterus:-
 - AVF, average size uterus measuring about: 8x4x4cm in its longitudinal, transverse & AP diameters respectively.
 - The myometrium shows homogenous texture with no focal lesion
 - The endometrial line is 4mm.
- Ovaries:-
 - Both ovaries are average size with no focal lesion
- No gross adnexal soft tissue masses or cysts.
- No free fluid in the Douglas pouch.

Conclusion:-

Unremarkable study

Signature Dr . Nermien Ramadan
PM 4:35 2025/07/21



Patient's Name:

Amal H₂

Fattah El-Sayed

Date:

2026-05-16

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,

ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- ☐ Known case of left breast cancer.
- ☐ Left upper lobar subpleural reticulations likely post therapeutic sequel
- ☐ No consolidation or collapse.
- ☐ No hilar or mediastinal masses or lymph node enlargement.
- ☐ No pleural or pericardial collection.
- ☐ Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- ☐ Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles. No hepatic focal lesion.
- ☐ Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- ☐ Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- ☐ The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- ☐ Normal appearance of the pelvic organs with no pelvic masses or collections.
- ☐ No abdominal or pelvic lymphadenopathy.
- ☐ No ascites.

Opinion:

- ❖ No hepatic or pulmonary metastatic lesions.



Patient's Name:

Amal He

Fattah El-Sayed

Date:

2026-05-16

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- o Known case of left breast cancer.
- o Left upper lobar subpleural reticulations likely post therapeutic sequel
- o No consolidation or collapse.
- o No hilar or mediastinal masses or lymph node enlargement.
- o No pleural or pericardial collection.
- o Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- o Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles. No hepatic focal lesion.
- o Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- o Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- o The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- o Normal appearance of the pelvic organs with no pelvic masses or collections.
- o No abdominal or pelvic lymphadenopathy.
- o No ascites.

Opinion:

- ❖ No hepatic or pulmonary metastatic lesions.



राष्ट्रीय कैंसर संस्थान
एन.सी.सी. इन्स्टीट्यूट ऑफ मेडिकल रिसर्च
एन.सी.सी. इन्स्टीट्यूट ऑफ मेडिकल रिसर्च

C-21-11-12

MDT FORM

Name	Dr. P. P. P.
Age	45
MRN	11111111111111111111
ID	11111111111111111111
Co	11111111111111111111
morbidity	11111111111111111111

Clinical Data

Bx:	IPCC II	SM:	lt UoQ
IHC:		Mets Workup:	None
ER:	B		
PR:	B		
Herts:	156		
K17:	156		

COMMENT:

1 CEST

He MDT

MDT OUTCOME & PLAN

CEM should be post 1/2 hrs 2.5 x 2.5
as post Amok A - P up front 2.5 x 2.5
CBS, SLNB

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

42/11/2
2011

10/11/2

10/11/2



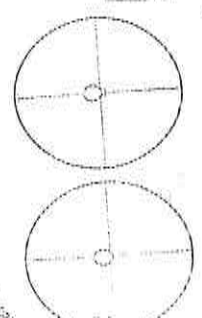
राष्ट्रीय कर्करोग नियंत्रण केंद्र
National Cancer Control Center

C-CL-11-42

MDT FORM

Name	के.डी.एम. पटेल
Age	45
MRN	017-CLNCC-000
ID	0000000000
Co morbidity	NAD

4/2
4/2
4/2



ST-4/2/2

Clinical Data

Bx:	IPCL	SM:	lt Noq
IHC:		Mets Workup:	None
ER:	8		
PR:	8		
Hist:	136		
KIV:			T ₂ N ₀ M ₀ (CR)
COMMENT:	① CEST The re-MDT		
MDT OUTCOME & PLAN			

→ CEST should JOD post 3 wks 2.5 x 2.5 cm
→ as Rptd around A → Rptd around S
CRBS, SLNB

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

10/12